



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان محمد احمد جوده
المحافظة : الفيوم الحى / المدينة : طاميه
رقم البطاقة : 28601152302605 رقم التليفون : 01124480161
العنوان : فانوس
العمر (تاريخ الميلاد) : 15-01-1986 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: خان محمد احمد جوده National ID: 28601152302605	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

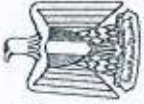
نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٣/٠١ ٢٨٦.١١٥٢٣.٢٦.٥

فلاحه

متروحة

مسلمة

عزت فحفي اساعيل محمد

البطاقة سارية حتى ٢٠٢٣/٠١/٢٩



جمهورية فلسطين العربية
مجلس إدارة

خان

محمد احمد جوده

عزبة الحجريه - قنوس

مركز ظافية - القويم

٢٨٦.١١٥٢٣.٢٦.٥



JU7600824



MDT FORM

14/ 9 / 2025 G

Name	هنا م م احمد	MRN	1514 / C.C
Age	36 y.	ID	1514.11092.07.0
Menopausal status	premen	Phone	01124480161
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX:

± DC II

ER:

8/8

PR:

6/8

HER 2:

2+

Ki67:

10-15%

Sonomammography:

SRU: Not Amplified

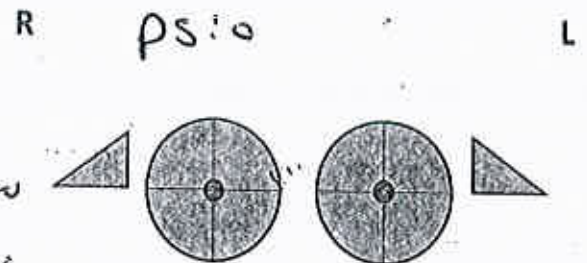
premenstrual for Breast

Mets Workup:

CT2, No. No

Pr BCS on 29/5/2022

T2, No 0/3, No



TNM

Recommendations:

Rec adv & LTC

st. Zol + TAM on 10/2022

J. MDT (to shift to AI & Zoladex as per the report)

2. 1/2 35 on diagnosis

MDT OUTCOME & PLAN

9/11

2. AI + Zoladex x 5 years
(Femara)

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

Signature of Medical Oncology

Signature of Radiology

Patient name: جنان محمد احمد

Date: 30-3-2022

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO D3T (Digital Breast tomosynthesis)
- Ultrasound examination using linear probe 6-12MHz

Report:

- ACR B: scattered fibro glandular parenchyma.
- Right UIQ faintly dense lesion.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass.
- Complementary us >>> two right breast 2 o'clock zone A,B, ill-defined isoechoic solid lesions seen with faintly vascularity seen measuring about 20 x 10mm and 6.5 x 8.5mm respectively.
- No left solid or cystic lesions
- No dilated ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LNs.

Opinion:

- Right UIQ low suspicious lesion for histopathological correlation (BIRADS 4A).
- Normal study of left breast (BIRADS 1).

Best regards,
Dr. Nehal Hamdy, FEBR, MSc



Women's Health Outreach Program

Name: HANAN MOHAMED AHMED

Date: 03.03.2022

Contrast Enhanced Full field Digital Mammography

Clinical summary:

Right pathological proven neoplastic lesion

Technique

Pre & post contrast Mediolateral oblique and CC views

Targeted High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibroglandular tissues Acr b
- No suspicious calcification or speculated mass lesions
- **Contrast enhanced images showed:** Nodular background parenchymal enhancement is seen. no mass or non-mass enhancement
- Normal skin/nipple/areolar complex.
- No pathological axillary LNs.

Right breast

- Scattered fibroglandular tissues Acr b
- **UIQ** irregular faintly dense lesion is seen.
 - **Contrast enhanced images demonstrated:** Nodular background parenchymal enhancement is seen. **UIQ** multiple enhancing mass lesions are seen (at least three lesions) occupying the middle and posterior thirds the largest measuring 2.48 x 2.2 x 0.8 cm and 1.9 x 1 cm with 2 cm in between. Another faintly enhancing lesion is seen in the **UOQ** measuring 1.2 x 1 cm both lesions are about 5 cm from nipple
- No pathological axillary LNs.

Conclusion:

- Left breast: normal study (BIRADS 1).
- Right breast: pathologically proven Multicentric neoplastic lesion as above described (BIRADS 6).

Best regards,
Heba Azzam, MD



Presidential Program for Women's health

Name: Hanan Mohamed Ahmed Goda

Date: 27-4-2022

Targeted right breast ultra sonography

Clinical summary: right breast UIQ pathologically proved malignant mass lesion .

Right breast:

- The enhanced UIQ nodule seen by CESM by U/S proved to be a small intramammary node with preserved fatty hilum ,it measures 5 mm and seen at 10 o'clock 4 cm from nipple .

Dr Nashwa A. El Kader



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m1162

Female	التنوع	اسم المريض	حضان محمد احمد
03/03/2022 10:46	العنوان	السن	
07/03/2022 09:13	تاريخ استلام العينة	تاريخ اخذ العينة	
	تاريخ التقرير	مستشفى القيوم العام	مستشفى القيوم العام
		طالب التحاليل	عالية

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

Sample received was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody.
Sample received was treated by: - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark G.
Sample received was treated by: - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER: strong nuclear staining of > 60 % of tumour cells (core biopsy)
PR: moderate nuclear staining of 40 % of tumour cells (core biopsy)
HER2/neu protein over expression: moderate continuous membrane staining of > 10 % of tumor cells (core biopsy)
Ki67: positive nuclear staining of 10 - 15 % of tumour cells (core biopsy)

Referral Lab :

Comment :

Conclusion :

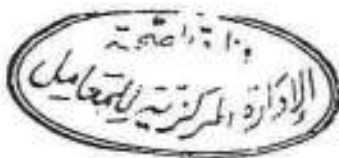
ER: POSITIVE (SCORE 8 / 8)
PR: POSITIVE (SCORE 6 / 8)
HER2/NEU PROTEIN OVER EXPRESSION: EQUIVOCAL (SCORE 2 +)
#KI67: THE PROLIFERATION INDEX OF THE TUMOUR IS 10 - 15 %

Pathologist:

د. نورا ماهر

Lab Director:

د. نورا ماهر



19 El-sheikh Rehan Street, Abeideen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144287292
website: http://www.healthlabs.net
email: pathologychi@yahoo.com
facebook: قسم الباثولوجي - الإدارة المركزية للمحليل

This Test is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m.1162

Female	: النوع	حنان محمد احمد	: اسم المريض
07/03/2022 09:41	: التاريخ		: السن
09/03/2022 09:44	: تاريخ استلام العينة		: تاريخ اخذ العينة
	: تاريخ التقرير	مستشفى القيوام العام	: طالب التحليل
		مستشفى القيوام العام	: عناية

Primary Sample :

Paraffin Block

Test :

HER2 by SISH

Comment :

Technique :

sample received was stained for HER2 gene and chromosome 17 using INFORM HER2 Dual Probe Cocktail technique in benchmark GX (VENTANA).

Result :

The ratio of Her 2 / chr 17 is less than 2 (core biopsy)

Referral lab :

Comment :

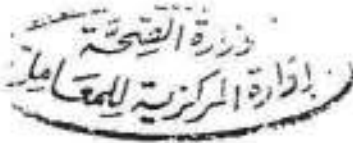
Conclusion :

Her 2 Status by SISH (Dual Ish Technique): Negative (not amplified Her 2 / chr 17 < 2)

Pathologist:

د. النعمان راضي

Lab Director: د. تروفا ماهر



19 El-sheikh Rehan Street, Abokdeen, Cairo
Tel. (165)7948544 - 7950096 Fax. (202)7948544
Mobile : 0114297292
website: http://www.healthlabs.net
email: pathologychl@yahoo.com
facebook: قسم الباثولوجي - الإدارة المركزية للمعامل

* This Test is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها



Ministry of health
Central health laboratories
Pathology department

Histopathology report

Report Number: 214

Female	: النوع	حنان محمد احمد	: اسم المريض
01/03/2022 14:39	: التاريخ استلام العينة	مستشفى القويم العام	: تاريخ اخذ العينة
06/03/2022 14:52	: تاريخ التقرير	مستشفى القويم العام	: طلب التحليل
			: عتلة

Primary sample :

east core biopsy

comment :

Gross examination :

cores of greyish white tissue (T.I)

Microscopic examination :

ection examined revealed fibrocollagenous tissue infiltrated by malignant growth displaying the morphology of infiltrating duct carcinoma

Referral Lab :

Comment :

Diagnosis :

BREAST CORE : PICTURE IS COMPATIBLE WITH INFILTRATING DUCT CARCINOMA , GRADE II

Pathologist:

Dr/Jehan Aziz

Lab Director: د/ تيريزا ماهر

19 El-sheikh Rehar Street, Abdeldeen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website: <http://www.healthlabs.net>
email: pathologychl@yahoo.com
facebook: قسم التشريخ الجوى - الادارة المركزية للمعمل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بالنسب لمدة 30 يوم من تاريخ استلامها

Page 1 of 1



Patient Name	: الأستاذة / حنان محمد أحمد جوده	Request No.	: 13006599
Patient No.	: 28601152302605	Request Date	: 29/05/2022
Path No.	: 1470/2022	Verification Date	: 11/06/2022
Sex	: Female	Financial Category	: أورام نادر السلام - نفقة الدولة
Age	: 36 Y	Department	: Out Patient
Referring Doc :		Registered by	: dr.Enas

Pathology Report

Clinical Data: Rt breast cancer

Nature of the specimen: WLE + SLNB

Gross Examination:

- Procedure: WLE measured 7x6x2cm, skin ellipse measured 4x0.5cm.
- Localization: Rt
- Tumor extent
 - o Number: An ill-defined firm gritty greyish white mass
 - o Size: 3x2x0.5cm
 - o Skin involvement: Grossly unremarkable
- Distance from margins: 1 cm from least margin
- Lymph nodes number and size: 3 nodules up to 2x1cm
- Frozen section: - Free margins. - Free (0/3) SLNs

Microscopic examination:

- Histological type: Invasive Ductal Carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1)
- Ductal carcinoma in situ: Wide 30% (comedo and non-comedo types)
- Lymph node involvement: Free (0/3) SLNs
- Resection Margins:
 - o Free

Diagnosis:

Rt breast cancer, WLE + SLNB:

- Invasive Ductal Carcinoma GII wit wide 30% Ductal carcinoma in situ, (comedo and non-comedo types)
- Free (0/3) SLNs
- o Free Resection Margins

Verified by :

dr.menna atef

Printing Date : 18/06/2022 09:02:25

Page 1 of 2

Printing By : Ali mostafa



Patient Name	: الأستاذة / حنان محمد أحمد جوده	Request No.	: 13003599
Patient No.	: 28601152302605	Request Date	: 29/05/2022
Path No.	: 1470/2022	Verification Date	: 11/06/2022
Sex	: Female	Financial Category	: أورام دار السلام - نفقة القوة
Age	: 36 Y	Department	: Cut Patient
Referring Doc :		Registered by	: dr.Enas

Pathology Report

o Stage: pT2, pN0

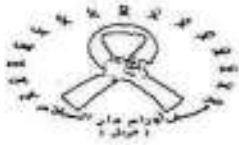
Prof. Lubna Omar
Dr. Merna Atef

Verified by :
dr.menna atef

Printing Date : 18/06/2022 09:02:25

Page 2 of 2

Pr. nting By : Ali mostafa



Name : Hanan Mohamed Ahmed

Date : 7/4/2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.
- D7, D10, D11 osseous hemangiomata.
- Right femoral head bone island.

OPINION:

- **Hepatomegaly.**
- **No metastatic pulmonary, hepatic or osseous deposits.**

Dr. Aya Deabes, MSc.

Dr. M. Gomaa, M.D.



Patient Name: Hanan Mohamed Ahmed Goda	Date : 06/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 38 Year(s)
Medical ID: 1284/2022	Pat. ID: 11920

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Indication:

Right breast conserving therapy

Report:

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- Scattered fibro glandular density (ACR B)
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
06/08/2024 1:07 PM

Name : Hanan Mohamed Ahmed

Date : 7/4/2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.
- D7, D10, D11 osseous hemangiomas.
- Right femoral head bone island.

OPINION:

- *Hepatomegaly.*
- *No metastatic pulmonary, hepatic or osseous deposits.*

Dr. Aya Deabes, MSc.

Dr. M. Gomaa, M.D.